

CONTENTS

Table of Contents

This manual (Rev. 2.4, July 2026) is organized into three parts and is fully compliant with the NCDHHS Records Management and Documentation Manual (RMDM, Effective July 8, 2025). Part 1 establishes foundational policies and compliance obligations across eleven sections, including new chapters on privacy/confidentiality and an expanded Medicaid documentation section. Rev. 2.1 added §10.7 Electronic Signatures (referencing NCGS Chapter 66, Article 40 — NC UETA) with explicit safeguards and system unavailability procedures. Rev. 2.2 clarified the organizational reporting structure in §1.4, establishing the QP as reporting to the Clinical Director, supervising staff per the Clinical Director's direction, and providing recurring compliance reports via new §1.4(a). Rev. 2.3 refined the QP responsibilities to explicitly include scheduling of clinical services, assessments, and PCPs in §1.4. Rev. 2.4 corrects the QP credentialing requirements to align with 10A NCAC 27G .0104; new §1.4(b) specifies acceptable NC credentials (full license, associate/provisional license, certification, or psychiatric nursing credential) and clarifies that a QP is not required to hold a full, unrestricted clinical license, but a bachelor's degree alone is not sufficient; §2.2 Staff Qualifications bullet updated to match. Part 2 details twenty-one step-by-step workflows that govern daily operations, including protocols for service orders/authorizations and record management/disclosure accounting. Part 3 provides nine customized forms and logs, including the Full Service Note Template, Comprehensive Clinical Record Content Checklist, and Accounting of Disclosures Log.

Standard Operating Procedures	5
1. Agency Overview & Governance	5
2. Human Resources & Staffing Requirements	7
3. Admissions, Discharges, and Transition Planning	8
4. Clinical Services, Assessments & Person-Centered Planning	8
5. Behavioral Management & Restraint	10
6. Health, Medication, & Nutrition Management	10
7. Education & Vocational Support	11
8. Incident Reporting & Response (Rule 108 / IRIS)	11
9. Facility, Safety, & Environmental Management	12
10. Medicaid Billing, Documentation Compliance & Record Management	12
11. Privacy, Security, Confidentiality & Access to Records	15
Actionable Workflows & Protocols	17

1. Admissions & Pending Record Protocol	17
2. Daily Direct Care Protocol (DCP)	17
3. Incident Response & IRIS Protocol	17
4. Behavioral Crisis & Restraint Protocol	18
5. Medication Administration Protocol	18
6. Medicaid Service Note & Documentation Protocol	18
7. AWOL / Missing Child Response	18
8. Suicide Risk & Self-Harm Management	19
9. Contraband & Room Search Protocol	19
10. PCP Implementation & Review	19
11. Transportation & Community Outing Safety	19
12. Grievance Process for Youth	20
13. Staffing Ratio & Awake Overnight Protocols	20
14. Medical Emergencies & Acute Illness Response	20
15. Educational Coordination & School Reintegration	20
16. Family Engagement, Visitation & Home Passes	20
17. Discharge & Transition Protocol	21
18. Infectious Disease & Bloodborne Pathogen Control	21
19. Emergency & Disaster Preparedness	21
20. Service Orders & Authorization Protocol	21
21. Record Management, Retention, Access & Disclosure Accounting	22
Customized Forms & Logs	22
1. Shift Change & Awake Night Watch Log	22
2. Contraband & Belongings Inventory	23

3. Physical Restraint & Debriefing Checklist	24
4. Home Pass & Medicaid Billing Exclusion Tracker	25
5. Emergency Drill & Environmental Safety Log	25
6. Employee SOP Acknowledgment	26
7. Full Service Note Template (Mandatory)	27
8. Comprehensive Clinical Record Content Checklist	27
9. Accounting of Disclosures Log	28
10. Version History	29

PART 1

Standard Operating Procedures

These eleven SOP sections define the foundational policies, governance, staffing, clinical, privacy, and compliance obligations that govern every aspect of program operation. Version 2.0 expands coverage to full RMDM (Effective July 8, 2025) compliance, including record retention, CDW data reporting, comprehensive clinical record content, CCA requirements, service orders/authorizations, and privacy/confidentiality under HIPAA and 42 CFR Part 2. Every staff member is responsible for knowing and adhering to these policies.

1. Agency Overview & Governance

Reference: 10A NCAC 27G .0100; NC Medicaid Managed Care Tailored Plan Requirements; RMDM Chapter 1 · Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · §1

1.1 Mission Statement. Well Spring Intervention LLC is dedicated to providing trauma-informed, high-quality supervised living environments for youth with severe emotional disturbances (SED), ensuring their safety and clinical growth. Our program philosophy is rooted in the belief that every child deserves a stable, predictable, and nurturing environment in which healing can occur. All staff actions, decisions, and interventions must align with this mission and reflect our core values of safety, dignity, accountability, and growth.

1.2 Licensing & Credentialing. The facility operates under a valid NC DHSR license (Level 3 Supervised Residential Group Home) and is credentialed as an In-Network Provider with the regional LME/MCO/Tailored Plan. The Executive Director maintains the original license on site, posts a current copy in a public area of the facility, and renews it prior to expiration. Any change in ownership, capacity, population served, or physical location requires prior written approval from DHSR and notification to the LME/MCO.

1.3 Corporate Compliance. Well Spring Intervention LLC maintains a Corporate Compliance Plan to prevent Medicaid fraud, waste, and abuse. All staff receive compliance training upon hire and annually thereafter. The Compliance Officer receives reports, investigates concerns, and coordinates with the LME/MCO and Medicaid Fraud Investigations Unit. Retaliation against good-faith reporters is strictly prohibited and itself constitutes a reportable offense.

1.4 Organizational Structure. The Executive Director holds overall authority and accountability. The **Clinical Director** is a licensed clinical professional with overall responsibility for the clinical program, including setting the clinical vision, approving clinical policies, and providing direction to clinical leadership. The **Qualified Professional (QP)** reports to the Clinical Director and is responsible for scheduling clinical services, assessments, PCPs, and day-to-day supervision of Associate Professionals (APs) and Direct Care Professionals (DCPs) **according to the direction of the Clinical Director.** The QP supervises staff in accordance with the Clinical Director's clinical guidance, programmatic priorities, and performance expectations, and shall escalate clinical concerns, staffing issues, and quality-of-care matters to the Clinical Director in a timely manner. An On-Call QP is available 24/7/365 for clinical decision-making. All staff report incidents, concerns, and operational needs through the documented chain of command.

1.4(a) QP Compliance Reporting to the Clinical Director.

In addition to clinical supervision, the QP shall provide **compliance reports** to the Clinical Director on a recurring basis, ensuring the Clinical Director has timely visibility into the facility's regulatory and documentation posture.

These reports include: (a) **monthly** summaries of service-note audit findings, late entries, and alterations (per §10.1, §10.3, §10.5); (b) **monthly** IRIS incident-report status including any restraint events, restrictive interventions, and follow-up actions completed (per §5.4, §8); (c) **quarterly** Clinical Record Content Checklist audit results using Form 8, with corrective-action plans for any deficiencies; (d) **quarterly** Accounting of Disclosures review using Form 9, including any 42 CFR Part 2 disclosures; (e) **quarterly** personnel-file audit results, sanctions reviews, and training-completion rates (per §2.5); (f) **annual** review of the electronic-signature safeguards in §10.7(a), conducted jointly with the IT vendor; and (g) **ad-hoc** immediate reporting of any reportable breach, complaint, licensing visit, or Medicaid audit. The Clinical Director reviews each report, signs acknowledgment, and directs corrective action as needed. The QP retains all compliance reports and Clinical Director acknowledgments in the facility compliance binder for the full record-retention period specified in §1.6.

1.4(b) QP Credentialing Requirements.

The Qualified Professional (QP) for this facility must meet the credentialing requirements of **10A NCAC 27G .0104**. A QP is not required to hold a full, unrestricted clinical license; however, a bachelor's degree alone is not sufficient. At a minimum, the QP must hold (a) a **master's degree in a human services field** from an accredited institution, plus (b) one of the following North Carolina credentials: (i) a **full clinical license** — LCSW, LPC, LMFT, Licensed Psychologist, Licensed Psychological Associate, or psychiatrist (MD/DO); (ii) an **associate or provisional license** — LCSW-A (Associate), LPC-A (Associate), LMFT-A (Associate), or LCAS-P (Provisional); (iii) a recognized **certification** — LCAS (Licensed Clinical Addiction Specialist), CCS (Certified Clinical Supervisor) with a master's degree, or CMSW (Certified Master Social Worker); or (iv) for the nursing discipline, a **Clinical Nurse Specialist (CNS)** or **Nurse Practitioner (NP)** with psychiatric/mental health certification. In addition, the QP must have at least **one year of full-time, post-master's supervised experience** in the delivery of mental health, developmental disabilities, or substance abuse services to the population served. The QP must also complete the **NC-DHHS-required QP training modules** prior to independent practice (or be enrolled and complete them within the probationary period). The facility shall verify and retain documentation of the QP's degree, current credential, supervised-experience hours, and QP training completion in the personnel file (§2.5). The QP is responsible for maintaining active credential status and reporting any lapse, sanction, or restriction to the Clinical Director within one business day.

1.5 Data Reporting & Consumer Data Warehouse (CDW).

The facility shall enroll all eligible individuals in the Consumer Data Warehouse (CDW) as required by DMH/DD/SUS. The QP shall ensure termination/discharge from CDW is completed after 60 consecutive days of no billable services. Statistical data required by DHHS, the General Assembly, and federally funded programs shall be submitted per contractual requirements. The QP shall maintain a log of CDW enrollment, updates, and terminations and review it monthly.

1.6 Record Retention and Disposition.

Clinical service records of minors shall be retained for **12 years after the minor reaches the age of majority (age 18)** — i.e., until age 30 — or longer if required by law or pending litigation. Clinical service records of adults shall be retained for **11 years after the date of the last encounter**, or longer if required. Personnel and administrative records are retained per the DHHS Records Retention and Disposition Schedule. Incident reports and investigatory files are retained per DNCR / Division of Archives and Records requirements.

Destruction of records not listed in an applicable schedule requires prior authorization from DMH/DD/SUS and the Division of Archives and Records via the "Request for Disposal of Unscheduled Records" form. **Record abandonment is strictly prohibited.** If abandonment is suspected, the facility shall notify the payor entity, relevant accrediting organizations, and DHHS, and shall cooperate fully with any investigation. Violations are subject to legal sanctions under HIPAA, 42 CFR Part 2, and the NC General Statutes.

2. Human Resources & Staffing Requirements

Reference: 10A NCAC 27G .0203 & .5600; NC Medicaid CCP 8C; RMDM Chapter 1 · Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · §2

2.1 Staffing Ratios. Minimum staffing ratios are mandated at all times to ensure resident safety and adequate supervision. Day and evening shifts maintain a 1:4 staff-to-resident ratio (awake and on-site), while the overnight shift maintains a 1:8 ratio (awake — no sleeping permitted). Ratios may be increased based on PCP acuity, behavioral incidents, or 1:1 supervision orders. The QP is responsible for monitoring ratios and adjusting assignments to maintain compliance at all times.

Shift	Minimum Ratio	Status	Notes
Day (7a-3p)	1 : 4	Awake / On-site	May increase per PCP acuity
Evening (3p-11p)	1 : 4	Awake / On-site	May increase per PCP acuity
Overnight (11p-7a)	1 : 8	Awake (No sleeping)	15-minute visual checks required

2.2 Staff Qualifications.

QPs: Master's degree in a human services field plus a recognized NC credential (full license, associate/provisional license, certification, or psychiatric nursing credential) and at least one year of full-time, post-master's supervised experience per §1.4(b) and 10A NCAC 27G .0104. A bachelor's degree alone is not sufficient. NC-DHHS QP training modules must be completed prior to independent practice.

APs: Bachelor's in human services with at least one year of relevant experience.

Direct Care Professionals (DCPs): High school diploma or GED with at least one year of mental health experience.

2.3 Background Checks (Prior to Unsupervised Contact).

NC SBI fingerprint criminal background check.

Health Care Personnel Registry check.

DSS Child Abuse and Neglect Registry check (every state of residence in prior 5 years).

Motor Vehicle Record (MVR) for staff who transport residents.

Re-checks completed annually and upon reasonable suspicion.

2.4 Mandatory Training (Prior to Independent Duty; Annual Refreshers).

CPR / First Aid (annually)

NCI or CPI restraint and de-escalation (annually)

Medication Administration (RN-delegated, annually)

Bloodborne Pathogens (annually)

Trauma-Informed Care (annually)

Rule 108 incident reporting (annually)

Adolescent development, C-SSRS suicide risk assessment, and person-centered planning (orientation + quarterly)

2.5 Personnel Records.

Personnel files shall contain: position descriptions; education verification; licensure and credentials; continuing education and training records; clinical and administrative supervision documentation; supervision plans; sanctions reviews from professional boards; documented review of the NC Health Care Personnel Registry; and criminal

background check results. Records shall be retained per applicable DHHS retention schedules. The QP audits personnel files annually to ensure completeness and compliance with RMDM Chapter 1.

3. Admissions, Discharges, and Transition Planning

Reference: 10A NCAC 27G .5604; NC Medicaid CCP 8C; RMDM Chapters 2 & 5 · Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · §3

3.1 Admission Criteria. The program serves youth with a primary mental health or behavioral diagnosis requiring supervised living, who are medically stable, and whose clinical needs can be safely met in a Level 3 group home setting. Exclusions include active psychosis requiring Involuntary Commitment (IVC), medical instability, or fire-setting that cannot be safely managed. The QP reviews each referral packet and documents the admission decision.

3.2 Admission Process — Pending vs. Full Records.

Upon initial presentation for screening, a **Pending Record** is created. This contains initial screening information, consultation and administrative coordination, and court-ordered evaluations that do not yet result in active enrollment. If the individual is enrolled in active services, the Pending Record shall be converted into a **Full Clinical Service Record** using the same record number, with all pending documentation merged. On the Day of Admission, the QP obtains signed consents (treatment, medication, transportation, ROI, photographs), the DCP conducts a respectful property search and secures medications, the QP administers the C-SSRS screening and documents immediate clinical status, and an RN visit is scheduled within 72 hours.

3.3 Full Clinical Service Record — Required Elements.

Upon conversion to a Full Record, the chart must contain all applicable elements enumerated in the Comprehensive Clinical Record Content Checklist (Form 8). Categories include: Consents; Demographics and Emergency Information; Health History (including DSM-5-TR/ICD-10 diagnosis and documentation of medication allergies, adverse reactions, and **absence of known allergies**); Medications and Labs; Rights and Restrictive Interventions; Assessments (CCA by a licensed professional); Planning (PCP with MID, service plan, signed service order); Discharge information; Referral information; Service notes/grids; Incidents (filed separately; occurrence noted in record); Disclosures and Legal documents; 42 CFR 2.22 summary for SUD; Accounting of Disclosures; and incoming/outgoing correspondence.

3.4 Discharge & Transition.

Transition planning is initiated at admission and reviewed every 30 days. Discharge may be planned or unplanned. The Discharge Summary is completed within **7 calendar days** of discharge (exceeds RMDM's 30-day requirement), and includes: reason for admission, course/progress, condition at discharge, recommendations, final diagnoses, and dated signatures. Medications are transferred to the guardian with a signed transfer form, belongings are inventoried, and the discharge summary is provided to the guardian, receiving provider, and LME/MCO.

3.5 Administrative Closure.

When a QP or clinician leaves employment without completing required discharge documentation for individuals meeting discharge criteria, the supervisor shall process the discharge. Each administratively closed record shall be audited per entity policy to ensure all billed services are properly documented. If audit reveals unmet documentation requirements, all services billed without proper documentation shall be adjusted back to the payor per 42 CFR 401.305. Documentation of the administrative closure shall be placed in the service record.

4. Clinical Services, Assessments & Person-Centered Planning

Reference: 10A NCAC 27E .0300; NC Medicaid CCP 8C; RMDM Chapter 4 · Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · §4

4.1 Comprehensive Clinical Assessment (CCA).

A CCA performed by a licensed professional (QP) is required **prior to service delivery**, except in crisis/emergency situations or when a current CCA (with no substantive change) is on file. The CCA must contain: (1) presenting problems, source of distress, precipitating events, associated symptoms; (2) chronological general health, past trauma, behavioral health history (MH/SU including tobacco), treatment history and response; (3) current medications for medical, psychiatric, and SUD treatment, including past ineffective medications or significant side effects; (4) review of biological, psychological, familial, social, developmental, and environmental dimensions identifying strengths, needs, and risks; (5) evidence of beneficiary and LRP participation; (6) analysis and case formulation; (7) **ASAM level of care determination** when substance use disorder is present; (8) DSM-5-TR diagnoses (MH, SUD, IDD, physical health, functional impairment); (9) recommendations for additional assessments, services, supports, or treatment; and (10) dated signature of the licensed professional completing the assessment.

For children and youth, the CCA must address Child and Family Team involvement; if new to services, recommend a Child and Family Team meeting; assess strengths of the child/youth and family (preferably with a strengths-based tool); and utilize IEPs and psychological testing reports if available. **Reassessment** is required when clinically indicated (new behaviors, unmet needs, annual PCP review, or when the service definition requires). If reassessment results in a diagnosis change, a written report is required; if refinement only, a clinical note is sufficient.

4.2 Medical Necessity Determination.

Medical necessity is established by the QP (or licensed/certified professional) based on diagnostic criteria and established best practice guidelines. The CCA and subsequent assessments shall support medical necessity for each service delivered. Medical necessity must meet applicable Medicaid and State-funded policy requirements (NC Medicaid Behavioral Health CCPs; DMH/DD/SUS State-Funded Service Definitions). The QP documents medical necessity in the PCP.

4.3 Person-Centered Plan (PCP) Development.

Within **30 calendar days** of admission, the QP facilitates a PCP meeting including the youth, guardian, LME/MCO representative, and other supports identified by the family. The PCP documents strengths, needs, goals, specific interventions, and a crisis plan. The PCP is signed by all participants, reviewed every 90 days thereafter, and updated via addendum for significant clinical changes (new diagnosis, medication change, major incident, or living arrangement change). Staff implement PCP goals daily and document progress in shift notes. The QP creates a "Staff Cheat Sheet" summarizing goals, triggers, and interventions for each youth, and staff sign the PCP Acknowledgment Form.

4.4 Service Orders.

Service orders are required for all Medicaid and State-funded services that require them. The service order is indicated by the appropriate professional's signature on the PCP. If a separate format is used, a separate service order is required. **Verbal orders:** treatment may proceed based on a verbal order; the verbal order must be documented in the service record on the date given, specifying the date, who gave the order, who received the order, and each distinct service ordered. Documentation must reflect why a verbal order was obtained in lieu of a written order. The appropriate professional must countersign the order with a dated signature **within 72 hours** of the verbal order.

4.5 Service Authorization.

Requests for authorization are required prior to initiation or continuation of services as per State-funded service definitions, Medicaid CCPs, or the authorizing entity's UM policy. The facility shall notify the authorizing entity when an individual changes providers or ends a service, and end-date reporting requirements must be followed. Service authorizations and reauthorizations are not required to be maintained in the clinical record but shall be available for audit purposes if requested.

5. Behavioral Management & Restraint

Reference: 10A NCAC 27G .0209; CMS Mental Health Parity Rules; RMDM Chapters 2 & 6 · Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · §5

5.1 Philosophy. Well Spring Intervention LLC employs a trauma-informed, positive behavioral interventions model. We recognize that behavior is communication, and our role is to teach and reinforce safer, more adaptive skills rather than punish behavior. Corporal punishment, withholding meals or sleep, social isolation as punishment, humiliation, and degradation are strictly prohibited and constitute reportable abuse under Rule 108.

5.2 Behavioral Support Plan (BSP).

Developed by the QP within 14 calendar days of admission, based on a Functional Behavioral Assessment (FBA). The BSP includes proactive strategies, replacement skills, de-escalation steps, and crisis responses. All staff review and sign the BSP Acknowledgment Form before working independently with the youth.

5.3 Restraint.

Restraint is an absolute last resort, used only when a youth is in imminent danger of serious harm to self or others and after less restrictive de-escalation has failed. Mechanical, chemical, and seclusion restraints are prohibited. Only NCI/CPI-trained staff may apply restraint, using the least restrictive hold. **Prone (face-down) positions are prohibited.** Post-restraint medical check (within 1 hour), debriefing (within 24 hours), and guardian notification (within 1 hour) are required. All restraints are reported in IRIS.

5.4 Restrictive Intervention Documentation.

Written notifications, consents, approvals, and other documentation per 10A NCAC 27E .0104(e)(9) are required whenever a restrictive intervention is used as a planned intervention. Any planned restrictive interventions must be included in the youth's service plan per 10A NCAC 27E .0104(f). Documentation in the service record must meet requirements of 10A NCAC 27E .0104(g)(2) and (g)(6), including rights restrictions (G.S. §122C-62(e)) and use of protective devices.

6. Health, Medication, & Nutrition Management

Reference: 10A NCAC 27G .0209; NC Nursing Practice Act; RMDM Chapters 5 · Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · §6

6.1 Medical Care.

Each resident has an identified Primary Care Physician (PCP) and psychiatrist upon admission. A complete medical examination is conducted within 30 days of admission and annually thereafter. Medical history (including immunizations, allergies, and current prescriptions) is maintained and updated at every visit. **Immunization compliance** for children is documented per NCGS §130A-152. Physician's directions for management of any identified medical conditions must be in the record.

6.2 Tuberculosis (TB) Screening.

Applicable if the facility receives SAPTBG funds for treatment services. Screenings must query: medical treatment in the past 3 months; current residence (jail, streets, shelter, etc.); history of TB tests; and physical

symptoms (night sweats, prolonged cough, shortness of breath, unexplained weight loss). Positive responses require referral to the local county health department or medical practitioner for follow-up testing and care. Completed screening and follow-up are documented in the service record.

6.3 Medication Management.

All medications are stored in a double-locked cabinet/cart. Controlled substances are counted and documented at every shift change. Medications are administered only by RN-delegated staff who have completed NC Medication Administration training. Staff follow the "5 Rights" (right youth, right med, right dose, right route, right time) and document administration on the MAR **immediately** (never retrospectively). Medication errors, refusals, and adverse reactions are reported to the RN and QP immediately, documented on the MAR, and entered into IRIS as required.

6.4 Nutrition.

Meals follow USDA guidelines. Special diets are accommodated with provider documentation. Menus are posted and retained 30 days. Food is never withheld as a consequence. Staff preparing food maintain current food handler certifications.

7. Education & Vocational Support

Reference: NC General Statutes (Education of Homeless Children); IDEA; RMDM Chapter 4 · Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · §7

7.1 Education Coordination.

The QP serves as Education Liaison for every youth in the program. Within 10 calendar days of admission, the QP contacts the Local Education Agency (LEA) — specifically the McKinney-Vento liaison and the EC Director — for enrollment, transportation, and service continuity. The QP signs an ROI for school communication, attends all IEP meetings, advocates for an aligned Behavioral Intervention Plan (BIP), and tracks daily school attendance. School refusal is treated as a clinical issue addressed through the PCP, not as a behavioral violation.

7.2 Vocational Support.

For youth age 14 and older, the PCP includes vocational and independent-living goals aligned with the transition plan. The QP coordinates with Vocational Rehabilitation and community-based employment programs to support the youth's transition to adulthood and post-discharge stability.

8. Incident Reporting & Response (Rule 108 / IRIS)

Reference: 10A NCAC 27T (Rule 108); RMDM Chapter 6 · Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · §8

8.1 Reportable Incidents.

The following events are reportable under Rule 108 and must be entered into the Incident Reporting and Investigative System (IRIS): death (including suicide attempts resulting in death), serious injury, alleged abuse or neglect, sexual abuse allegations, use of physical restraint, AWOL/missing child, medication errors, suicidal ideation or self-harm, and any other event meeting the regulatory definition of a reportable incident. When in doubt, staff report.

8.2 Reporting Timeframes.

Staff notify the On-Call QP **immediately** upon discovery of a reportable incident. The QP enters the IRIS report **within 24 hours** of discovery. Allegations of abuse/neglect and missing child trigger **immediate (within 15**

minutes) notification to DSS and law enforcement as appropriate. The guardian is notified **within 1 hour** of incident discovery (unless doing so would compromise an investigation). Verbal notifications are followed by written documentation.

8.3 Investigations.

Internal investigations are initiated within 24 hours and completed within 14 calendar days. Investigations include interviews, video review, documentation review, and a written narrative. A Corrective Action Plan (CAP) is developed for systemic or individual performance issues and uploaded to IRIS.

8.4 Incident Documentation — Separate Filing.

The occurrence of an incident shall be recorded in the service notes. **The completed incident report shall NOT be referenced or filed in the clinical service record.** It shall be filed separately in administrative files. All incident reports are maintained per the Records Retention and Disposition Schedule.

8.5 Abuse/Neglect/Exploitation Reporting.

Relevant facts must be documented in the service record, including reports made by the individual and actions taken. Reporting duties follow G.S. §7B-301 (child abuse), G.S. §108A-102 (disabled adult), and 10A NCAC 27G .0604. The completed incident report is filed separately from the clinical record as required by RMDM Chapter 6.

9. Facility, Safety, & Environmental Management

Reference: 10A NCAC 27G .0600; NC Fire Code; RMDM Chapter 3 · Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · §9

9.1 Environment.

The facility maintains a safe, clean, home-like environment. Maximum occupancy is two youth per bedroom, with each youth having individual storage space for personal belongings. Indoor temperatures are maintained between 68°F and 80°F. Hot water is regulated to ≤120°F to prevent scalding. Daily safety inspections are conducted by the DCP; weekly inspections by the QP. Hazards are corrected immediately or escalated to the Executive Director.

9.2 Drills & Inspections.

Monthly fire drills target evacuation under 3 minutes. Quarterly tornado drills use the interior safe room. Smoke detectors, fire extinguishers, sprinklers, and CO detectors are inspected monthly (staff) and annually (licensed contractors). Inspection records are maintained on the Environmental Safety Log (Form 5) for a minimum of 3 years.

9.3 Hazardous Materials.

Cleaning chemicals, sharps, tools, and medications are stored in locked areas. Safety Data Sheets (SDS) are maintained and accessible. No weapons are permitted on facility grounds at any time.

10. Medicaid Billing, Documentation Compliance & Record Management

Reference: NC Medicaid CCP 8C; CMS Documentation Guidelines; RMDM Chapter 6; NCGS Ch. 66 Art. 40 (NC UETA); E-SIGN Act (15 U.S.C. § 7001 et seq.) · Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · §10

10.1 Service Notes — General Requirements.

Service notes are the supporting evidence of individual outcomes and must be individualized. **Photocopying or repeating notes verbatim from a prior date or another individual's record is strictly prohibited.** When referencing another individual, use initials, record number, or role (e.g., "sibling") rather than full name unless clinically pertinent and reviewed prior to release. **Timelines:** facility-based services (this program) must be documented at time of service or **within 1 standard business day (not to exceed 24 hours)**. **Late entries** are entries after the required timeframe, must be marked "Late Entry" with the date of entry and the date the service was provided (e.g., "Late Entry made on 7/15/26 for service provided on 7/10/26"), and require a dated signature. Entries exceeding the timeline can only be billed up to **7 business days** beyond the service date; after 7 days, the note is not billable.

10.2 Full Service Note — Required Content.

Every shift/service note must include:

Name of the individual receiving the service (on each page).

Service Record Number issued by the payor along with the **Medicaid Identification Number (MID)** (as applicable), or unique identifier issued by the entity.

Full date the service was provided (month/day/year).

Name of the service provided.

Type of contact: In person, Telehealth, Telephonic, or Collateral.

Place of service.

Purpose of the contact (tied to specific goals in the PCP/ISP).

Description of the interventions, treatment, and support provided (specific and individualized).

Duration: total time spent performing the service (must include active engagement). For this residential per diem service, total time is not required for billing, but interventions must accurately reflect treatment for the event/per diem.

Effectiveness of the intervention(s) and the **individual's response/progress** toward goal(s).

Authentication: electronic signature date stamp and credentials (or handwritten signature with date and credentials). **Use of cursive font in a Word document is NOT a valid electronic signature.** Pre- or post-dating signatures is prohibited.

Additional requirements per Medicaid CCP or State-funded service definition.

Program, Intervention, and Evaluation (PIE) elements are part of the full service note. All required elements must be included for billing and audit purposes. Use Form 7 (Full Service Note Template) as the standardized format.

10.3 Shift Notes (Twenty-Four-Hour Services).

For 24-hour facilities, there must be a note for each shift, and **coverage hours for each shift must be clearly identified** in each note. If no intervention occurs (e.g., youth asleep or at school), the shift note shall reflect the care, oversight, support, and non-treatment events during the shift. When more than one staff person is providing services for a shift, **only one staff is required to document and sign the shift note**; however, the note must **identify the names and titles/positions of other staff persons present** to demonstrate staffing ratios are met.

10.4 Modified Service Notes & Grids.

Modified notes may be used only for services where explicitly allowed in CCP/State Service Definitions (e.g., Respite). Modified notes must include: Name, Service Record #/MID/Unique ID, Service provided, Date, Duration, Goal addressed/tasks performed, and Full dated signature/credentials (or initials if full signature is on the page).

Grids may be used only for specified services (e.g., Residential Supports — NC Innovations, Respite). Grids must include: Name, Service Record #/MID/Unique ID, Service, Date, Duration, Goals/Interventions, and Full dated signature/credentials (or initials if full signature is on the page). For NC Innovations, grids must include an intervention key, progress key, and a comment section with dated entries. **This facility will primarily use full**

shift notes. If using grids for Respite, the grid must meet all requirements above.

10.5 Alterations to Service Documentation.

The original entry must not be deleted or altered so it is not clearly legible in its original context. Corrections must be made by the person who initially authored the entry, with an explanation of the correction. If that is not possible, the person updating the documentation must reflect the needed change and the rationale. Any change must be **signed and dated**. The revised/corrected entry must be in proximity to the original entry. EHR systems with tracked changes and timestamps satisfy these guidelines. **Alterations exceeding 7 business days beyond the service date are not billable.**

10.6 Authenticating Service Documentation.

All entries are validated via signature (EHR or paper) with signature date, credentials, degree, licensure, and/or title. **Initials are only permitted** for updates/corrections or for notations entered on a service grid (where full signature is noted on the grid signifying appropriate initials). **Pre- or post-dating signatures in any form is prohibited.** **ADA Accommodations:** if an individual has a documented medical/physical reason (per ADA) for not being able to sign, another means for providing signature is acceptable; the process must be incorporated in facility policy. **Unavailable Original Author:** if the original author is no longer available to sign or amend an entry, a notation reflecting this shall be documented in the record and signed/dated by the appropriate party. **Rubber Stamps:** only used for medical/physical reasons with ADA accommodations; if unable to use stamp, the individual may authorize someone of their choosing in writing.

10.7 Electronic Signatures.

Electronic signatures are permitted for authenticating service documentation, consents, treatment plans, and other records governed by this manual, in accordance with the **North Carolina Uniform Electronic Transactions Act (NCGS Chapter 66, Article 40)** and the federal E-SIGN Act (15 U.S.C. § 7001 et seq.). An electronic signature is an electronic sound, symbol, or process attached to or logically associated with a record and executed or adopted by a person with the intent to sign the record. Where electronic signatures are used, they carry the same legal weight and enforceability as handwritten signatures for transactions conducted electronically. **Use of a cursive font in a word-processing document does not constitute a valid electronic signature;** the signature must be applied through the facility's authenticated EHR/EMR system or an approved secure signing platform that captures the signer's identity, timestamp, and intent. Each electronic signature shall be uniquely linked to the signer, render any subsequent modification detectable, and include an auditable timestamp reflecting the date and time of execution.

10.7(a) Safeguards.

The facility shall implement and maintain reasonable administrative, technical, and physical safeguards to ensure the integrity, confidentiality, and non-repudiation of electronic signatures, consistent with the HIPAA Security Rule (45 CFR Part 164 Subpart C), 42 CFR Part 2, and NCGS § 66-40(d). Required safeguards include: (a) **unique user credentials** for each authorized signer, with shared logins strictly prohibited; (b) **multi-factor authentication** where technically feasible; (c) **automatic session timeout** after no more than 15 minutes of inactivity; (d) **encrypted transmission** (TLS 1.2 or higher) and **at-rest encryption** of all signed records; (e) **immutable audit trails** capturing the signer's identity, date, time, IP address or device identifier, and the document version signed; (f) **role-based access controls** limiting signature authority to staff with appropriate licensure, credentials, and documented training; (g) **immediate revocation** of credentials upon termination, role change, or suspected compromise; and (h) **annual review** by the QP and IT vendor to verify continued compliance. Lost, stolen, or shared credentials must be reported to the QP within one business day; the facility shall deactivate the compromised credentials and investigate the scope of any unauthorized signatures before reissuing access.

10.7(b) System Unavailability Procedures.

In the event the EHR/EMR system or approved electronic signing platform is unavailable — whether due to planned maintenance, internet outage, vendor outage, cyber-incident, or natural disaster — staff shall revert to paper-based documentation and handwritten signatures with date, credentials, and title. The QP or designee shall declare a **Documentation Continuity Event**, notify all shifts in writing (or by phone tree if email is unavailable), and distribute blank copies of the required forms (Shift Note, MAR, Restraint & Debriefing Checklist, Incident Report). Staff shall legibly sign and date each paper entry in blue or black ink. When the electronic system is restored — and in no event later than **72 hours** after the original entry — the originating staff member (or, if unavailable, the on-duty QP) shall transcribe the paper entry verbatim into the electronic record, mark the entry as *"Late Entry — System Unavailability (date/time of restoration)"*, reference the original paper document by scan or filename, and electronically sign the transcribed entry. The original paper document shall be scanned and attached to the electronic record, then retained as a source document per the record retention policy (§1.6) for the full retention period. Paper entries that cannot be transcribed within **7 business days** must be reviewed by the QP, who shall document the reason for delay and the corrective action taken. The QP shall maintain a log of all Documentation Continuity Events, including the start/end time, cause, records affected, and verification that all paper entries were transcribed and electronically authenticated.

10.8 Service Authorizations & End-Date Reporting.

The facility shall maintain service authorizations/reauthorizations in a separate audit file (not required in clinical record but available upon request). When a youth changes providers or ends a service, the facility must notify the authorizing entity per their specified protocol. End-date reporting shall follow the authorizing entity's requirements.

10.9 Billing.

The program bills Medicaid on a daily per diem basis. Billing is suspended for any day the youth is hospitalized, detained, or on a home pass exceeding **24 consecutive hours**. The Home Pass & Medicaid Billing Exclusion Tracker (Form 4) documents all absences and corresponding billing action. Billing errors are corrected promptly and reported to the LME/MCO as required. Records are released only with a signed ROI or as required by law, court order, or regulatory audit.

11. Privacy, Security, Confidentiality & Access to Records

Reference: RMDM Chapter 3; HIPAA Privacy/Security Rule; 42 CFR Part 2; HITECH Act; NCGS §122C-52 through 122C-56 · Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · §11

11.1 Privacy and Security Policies.

The facility shall maintain comprehensive policies addressing how information is recorded, stored, retrieved, disseminated, and protected against loss, theft, destruction, unauthorized access (breach), and natural disasters. Compliance with the HIPAA Privacy and Security Rules, the Omnibus HIPAA final rule, the HITECH Act, and ARRA is mandatory. Policies shall address electronic communication (email, voice messaging, SMS/text speak, EHR usage). Where multiple requirements exist, the **most stringent** applies, including professional codes of ethics.

11.2 Safeguards.

Formal policies and procedures shall reasonably protect against unauthorized uses and disclosures of PHI, and against reasonably anticipated threats or hazards (42 CFR § 2.16). Secure storage: locked, climate-controlled areas with limited access; electronic records backed up daily and stored securely off-site.

11.3 Confidentiality.

Compliance is required with NCGS §122C-51 through 122C-56; 10A NCAC 26B; **42 CFR Part 2** (Confidentiality of Alcohol and Drug Abuse Patient Records); and NCGS §130A-143 (AIDS and related conditions). **Substance**

Use Information: requires the individual's written authorization before disclosure to other treatment providers (unless court order or allowable exception per 42 CFR Part 2). Absent written consent, entities must redact all identifying information from alcohol/drug records when sharing. **Care Coordination Exceptions:** under HIPAA and NCGS §122C-55(a), entities may share confidential information for coordination of care/treatment without written consent (requirements of NCGS §122C-55(a)7 apply); however, 42 CFR Part 2 still requires written authorization for SUD records.

11.4 Disclosure Documentation and Accounting.

Authorization must be in writing, voluntary, and informed (understanding what is exchanged, with whom, and for what purpose). Staff must verify the identity of the person requesting PHI and obtain required documentation/representations per Omnibus HIPAA rules. The **minimum necessary** standard (164.502(b)) applies. **Accounting of Disclosures** must be maintained for a minimum of **six years**. The accounting must include: name of the individual; medical record or ID number; date the information was released/disclosed; provider/entity/agency/individual to whom information was released; purpose of the release/disclosure; description of specific information released/disclosed; and name of person disclosing the information (recommended). All disclosures or re-disclosures of SUD treatment information must comply with 42 CFR Part 2; re-disclosure is prohibited except per NCGS §122C-53 through 122C-56. Releases/disclosures from external entities must be contained in the individual's service record.

11.5 Individual Access to Service Records.

Individuals and Legally Responsible Persons (LRPs) have the right to access information contained in their service record, per NCGS and DHHS provisions. **Limited Access:** if access is restricted, proper justification for restricting access must be clearly indicated in the service record. Individuals have the right to appeal such determination. Refer to Article 3, Part 1 (Client Rights), the Omnibus HIPAA Final Rule, and DHHS Privacy/Security policies.

11.6 Transporting Records.

Service records shall only be transported by designated individuals (hard copy, USB, tablet, CPU, etc.). When removed from premises, records shall be secured in a locked compartment and protected from unauthorized access. Policy must address what occurs if information is lost or stolen during transport, including immediate notification of the QP, Compliance Officer, and — if PHI is compromised — notification of affected individuals and DHHS per breach notification rules.

11.7 Storage and Maintenance.

Storage and maintenance are consistent with privacy/security principles. Electronic records self-warranty per NC State Archives guidelines. "Managing Public Records Produced by Information Technology Systems" guidelines apply. The QP conducts quarterly audits of storage security and access logs.

PART 2

Actionable Workflows & Protocols

These 21 protocols translate the SOP policies into clear, sequential, step-by-step actions for daily operations. Each protocol is designed to be referenced quickly during a shift and followed exactly. Deviations from protocol require QP approval and must be documented. Protocols 20 and 21 are new in Version 2.0 and address service orders/authorizations and record management/disclosure accounting per RMDM requirements.

1. Admissions & Pending Record Protocol

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 1: Admissions & Pending Record Protocol

Pre-Admission. QP reviews referral packet, verifies active Medicaid, schedules tour with guardian and youth.

Pending Record Creation. If individual presents for screening but is not immediately enrolled, create a Pending Record with screening/consultation info.

Day of Admission. QP obtains all signed consents (treatment, medication, transportation, ROI, photos). DCP conducts respectful property search and secures medications. QP administers C-SSRS and documents clinical status. Convert Pending Record to Full Clinical Record (merge all documentation).

Post-Admission. RN visits within 72 hours for medication delegation and nursing assessment. ISP/PCP meeting scheduled within 30 days.

2. Daily Direct Care Protocol (DCP)

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 2: Daily Direct Care Protocol (DCP)

Day Shift (7a-3p). Shift change handoff and controlled-substance med count. Wake youth per schedule with 15-minute visual checks. Breakfast and AM medications. School transportation. Daily chores.

Evening Shift (3p-11p). Shift change. Therapy and clinical groups. Chores, dinner, and PM medications. Free time and hygiene routine. 10:00 PM bedtime.

Night Shift (11p-7a). Shift change and med count. Conduct 15-minute line-of-sight room checks documented on the Night Watch Log. Awake at all times — no sleeping permitted.

3. Incident Response & IRIS Protocol

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 3: Incident Response & IRIS Protocol

Immediate. Ensure scene safety; call 911 if medical emergency or active danger. Notify the On-Call QP immediately.

Notifications. DSS immediately for abuse allegations. Guardian within 1 hour. LME/MCO per their requirements.

Reporting. QP enters the IRIS report within 24 hours using objective, factual narrative — no interpretations or labels.

Investigation. QP completes the internal investigation within 14 days and uploads findings and CAP to IRIS.

Separate Filing. Record the occurrence of the incident in the service note. File the completed incident report separately in administrative files — NOT in the clinical record.

4. Behavioral Crisis & Restraint Protocol

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 4: Behavioral Crisis & Restraint Protocol

Tier 1 — De-escalation. Use BSP coping skills: sensory items, breaks, verbal redirection, change of environment.

Tier 2 — Crisis. Clear the area of other youth. Call for staff backup. Use the NCI/CPI verbal continuum and physical distancing.

Tier 3 — Restraint. Only for imminent serious harm. Apply NCI hold; never use prone, mechanical, or chemical restraints. End the hold the moment danger passes.

Post-Restraint. Medical check within 1 hour. Notify guardian within 1 hour. Debrief with youth within 24 hours. File IRIS report.

5. Medication Administration Protocol

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 5: Medication Administration Protocol

Prep. Wash hands. Unlock med cart. Verify the "5 Rights" against the MAR: right youth, right med, right dose, right route, right time.

Administer. Verify youth identity. Pour medication into a labeled cup. Observe swallowing. Check the mouth for "cheeking".

Document. Initial the MAR immediately. Circle "R" if refused; notify RN and QP. Never pre-initial or backdate.

Errors. Stop, isolate remaining meds, call RN/Psychiatrist, monitor the youth, file IRIS, and notify the guardian.

6. Medicaid Service Note & Documentation Protocol

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 6: Medicaid Service Note & Documentation Protocol

Write the shift note by end of shift. No backdating — ever.

Mandatory Fields. Youth Name & Service Record # / MID. Full Date & Place of Service. Name of Service & Type of Contact (in-person, collateral, etc.). Purpose (ISP Goal # referenced). Objective Description of Interventions (factual, specific). Duration (for time-based services; for per diem, indicate shift coverage hours). Effectiveness & Youth Response/Progress. Full Signature with Credentials and Date.

Shift Notes. Identify coverage hours and names of all staff present for ratios.

Late Entries. Mark "Late Entry made on [date] for service on [date]" if after 24 hours.

Alterations. Correct with explanation, signed/dated; never obscure original. Alterations exceeding 7 business days are not billable.

Billing suspended for any day youth is away for more than 24 consecutive hours.

7. AWOL / Missing Child Response

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 7: AWOL / Missing Child Response

Discovery. Call the staff code for missing youth. Search the immediate area and check all cameras.

Notifications (within 15-30 minutes). Law enforcement, guardian, LME/MCO, and DSS (if applicable).

Documentation. File IRIS within 24 hours with a complete narrative.

Return. Conduct a medical check, contraband search, and trauma-informed debriefing. Notify all parties who were previously contacted.

8. Suicide Risk & Self-Harm Management

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 8: Suicide Risk & Self-Harm Management

Screening. C-SSRS at admission and immediately whenever threats or self-harm occur.

Safety. Remove hazards (ligatures, sharps, cords). Implement 1:1 line-of-sight supervision.

Clinical. QP assesses within 1 hour. If imminent risk: call 911 or initiate IVC. If moderate risk: update the Safety Plan and increase supervision.

Documentation. File IRIS. Staff sign the updated Safety Plan and acknowledge the increased supervision level.

9. Contraband & Room Search Protocol

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 9: Contraband & Room Search Protocol

Definition. Contraband includes weapons, drugs, unauthorized medications, lighters, chargers with frayed cords, and any item prohibited by house rules.

Routine. Conducted at admission and on return from any pass. The youth must be present.

Probable Cause. Requires QP approval. Two staff must be present. Document the basis for the search.

Execution. Search systematically (one area at a time). Safe items are locked in the safe and returned at discharge. Illegal items are turned over to law enforcement.

10. PCP Implementation & Review

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 10: PCP Implementation & Review

Development (30 Days). QP schedules the meeting with the team and drafts measurable, individualized goals.

Operationalize. QP creates a "Staff Cheat Sheet" summarizing each youth's goals, triggers, and interventions. Staff sign the PCP Acknowledgment Form.

90-Day Review. QP tracks expirations and schedules reviews. Updates progress, obtains signatures, and writes addendums for any significant change.

Service Order. The signed PCP serves as the service order for all services delivered.

11. Transportation & Community Outing Safety

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 11: Transportation & Community Outing Safety

Driver Prep. Valid license, clean MVR on file. Complete the daily vehicle safety checklist. Confirm first-aid kit and Medicaid cards are present.

Transport. Conduct a headcount before leaving and upon return. If transporting one youth, staff sits in the back seat. Seatbelts on at all times. Never leave youth unattended in a vehicle.

Outings. Tied to an ISP goal. Assess behavioral stability (no restraint in the last 4 hours). Carry the agency cell phone and emergency contact list.

12. Grievance Process for Youth

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 12: Grievance Process for Youth

Accessibility. Process is explained at admission. Blank forms and a locked grievance box are located in the common area. LME/MCO grievance numbers are posted.

Filing. Verbal or written. Staff assist immediately upon request. Place the grievance in the locked box within 1 hour.

Resolution. QP opens the box daily. Acknowledges within 24 hours. Provides written resolution within 7 days. Abuse allegations trigger an IRIS report and immediate DSS notification.

13. Staffing Ratio & Awake Overnight Protocols

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 13: Staffing Ratio & Awake Overnight Protocols

Ratios. 1:4 day and evening. 1:8 overnight. If a staff member calls out, the on-call system activates a replacement; the QP covers in-house if no replacement is available.

Awake Overnight. Sleeping is prohibited. 15-minute visual room checks are documented on the Night Watch Log. Hallway lights remain on. Engage nighttime wakers quietly and briefly — do not start conversations that escalate arousal.

14. Medical Emergencies & Acute Illness Response

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 14: Medical Emergencies & Acute Illness Response

Triage. Assess the situation. Call QP and/or RN. Err on the side of caution — when in doubt, transport.

Emergency. Call 911. Staff rides in the ambulance. QP calls in additional staff to maintain ratio at the facility.

Non-Emergency. Contact PCP for a same-day appointment. Take consent forms and Medicaid card.

Documentation. Guardian notified within 1 hour. IRIS filed. Billing suspended if the youth is hospitalized overnight.

15. Educational Coordination & School Reintegration

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 15: Educational Coordination & School Reintegration

Enrollment (5 Days). QP contacts the LEA McKinney-Vento liaison or EC Director. Signs ROI. Coordinates transportation.

IEP. QP/AP attends IEP meetings. Advocates for an aligned BIP. Files the IEP in the youth's chart.

Attendance. Morning check before transport. Log daily. Refusal is an ISP issue, not a punishable offense. Collect daily behavior reports from the school.

16. Family Engagement, Visitation & Home Passes

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 16: Family Engagement, Visitation & Home Passes

Visitation. Based on ISP/court orders. Approved visitor list is maintained in the office. Staff search bags before visits. Document all visits.

Home Passes. QP and guardian must approve. Pass medications are provided in a locked box with administration instructions.

Billing Exclusion. If the youth is away for more than 24 consecutive hours, Medicaid per diem is suspended. Notify the Billing Coordinator immediately.

17. Discharge & Transition Protocol

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 17: Discharge & Transition Protocol

Planning. Begins at admission. Trial home visits are conducted prior to discharge whenever possible.

Step-Down. Discharge meeting is scheduled. Community linkages (outpatient therapy, medication management, school) are established and confirmed before discharge.

Discharge Day. Medications are handed directly to the guardian with a signed transfer form. Belongings are inventoried. Discharge summary is completed within 7 days.

Administrative Closure. If QP/clinician leaves without completing discharge documentation, the supervisor processes the discharge and audits the record. Billed services without proper documentation are adjusted back to the payor per 42 CFR 401.305.

18. Infectious Disease & Bloodborne Pathogen Control

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 18: Infectious Disease & Bloodborne Pathogen Control

Universal Precautions. Treat all bodily fluids as infectious. Wear appropriate PPE (gloves, mask, eye protection) when exposure is possible.

Exposure. Wash the affected area with soap and water. Notify QP and RN. Go to Urgent Care within 2 hours. File IRIS. Offer post-exposure testing per protocol.

Outbreak. Two or more youth with identical symptoms = isolate, increase sanitation, and notify the County Health Department if the illness is reportable.

19. Emergency & Disaster Preparedness

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 19: Emergency & Disaster Preparedness

Fire. Evacuate immediately. Headcount at the rally point. Call 911. Do not re-enter. Monthly drills required.

Tornado. Move to the interior safe room. Turtle position. Quarterly drills required.

System Failure. Distribute flashlights. If heat loss exceeds 4 hours or water loss exceeds 8 hours, initiate the Emergency Relocation Plan.

Lockdown. Secure doors and windows. Hide. Silence phones. Do not open for anyone but law enforcement. Annual drills required.

20. Service Orders & Authorization Protocol

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 20: Service Orders & Authorization Protocol

Service Order. The PCP shall serve as the service order when signed by the appropriate professional. If a separate format is used, ensure a separate service order is signed.

Verbal Orders. If a verbal order is obtained, document in the record on the date given: date, who gave, who received, services ordered, and reason for verbal. The professional must countersign within 72 hours.

Authorization. Submit authorization requests to the authorizing entity prior to initiation or continuation per their UM policy.

End-Date Reporting. Notify authorizing entity immediately when youth changes providers or ends service. Follow entity-specified protocol.

Maintain authorization/reauthorization records in an audit-ready file (not in clinical record).

21. Record Management, Retention, Access & Disclosure Accounting

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 21: Record Management, Retention, Access & Disclosure Accounting

Retention. Minors — 12 years after age 18 (until age 30); Adults — 11 years after last encounter.

Personnel/Admin records per DHHS schedule. Records not on a schedule require DMH/DD/SUS authorization before destruction.

Record Abandonment. Strictly prohibited. Notify payor, accreditor, and DHHS if suspected. Violations are subject to legal sanctions.

Access. Youth/LRP may access records. If access is restricted, document justification in record. Appeal rights apply.

Accounting of Disclosures. Maintain Form 9 (Accounting of Disclosures Log) for each youth. Include: Name, Record #, Date disclosed, Recipient, Purpose, Description of info, Disclosing party. Retain for minimum 6 years.

Confidentiality. Comply with 42 CFR Part 2 for SUD records. Obtain written ROI for SUD disclosures. Minimum necessary standard applies.

Transport. Only designated staff; secured in locked compartment; documented policy for loss/theft.

PART 3

Customized Forms & Logs

The following nine forms and logs are the official documentation instruments for daily operations. Each form must be completed in ink or in the electronic record system, signed by the responsible staff member, and filed in the youth's chart or the facility's operational binder as indicated. Originals are retained per the record retention policy (12 years after the minor reaches age 18; 11 years for adults). Forms 7, 8, and 9 are new in Version 2.0 to address RMDM service-note, clinical-record-content, and disclosure-accounting requirements.

1. Shift Change & Awake Night Watch Log

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · Part 3 · Form 1: Shift Change & Awake Night Watch Log

Facility: Well Spring Intervention LLC **Date:** _____

Awake Overnight Room Checks (Every 15 Minutes)

Instructions: Initial each box to verify you visually saw the youth breathing and in their bed.

Time	Youth 1	Youth 2	Youth 3	Youth 4	Staff Initials
11:00 PM					
11:15 PM					
11:30 PM					
11:45 PM					
12:00 AM					
12:15 AM					
12:30 AM					
12:45 AM					
1:00 AM					
1:15 AM					
1:30 AM					
1:45 AM					
2:00 AM					
2:15 AM					
2:30 AM					
2:45 AM					
3:00 AM					
3:15 AM					
3:30 AM					
3:45 AM					
4:00 AM					
4:15 AM					
4:30 AM					
4:45 AM					
5:00 AM					
5:15 AM					
5:30 AM					
5:45 AM					
6:00 AM					
6:15 AM					
6:30 AM					
6:45 AM					

Shift Handoff Verification: Controlled substance count verified.

Off-Going Staff: _____ On-Coming Staff: _____

2. Contraband & Belongings Inventory

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · Part 3 · Form 2: Contraband & Belongings Inventory

Youth Name: _____ **Service Record # / MID:** _____ **Date:** _____

Search Type: ☐ Admission ☐ Return from Pass ☐ Probable Cause (QP Approval: _____)

Item Description	Quantity	Brought In / Found	Disposition (Kept / Safe / Guardian)	Staff Initials

Youth Acknowledgment: Belongings inventoried in my presence.

Youth Signature: _____ Staff Signature: _____

3. Physical Restraint & Debriefing Checklist

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · Part 3 · Form 3: Physical Restraint & Debriefing Checklist

Youth: _____ **Service Record # / MID:** _____ **Date:** _____ **Time Started:** _____

Time Ended: _____

Total Duration (Min): _____ **Technique:** _____

Pre-Restraint De-escalation Attempts:

☐ Verbal redirection ☐ Offered break ☐ Sensory item ☐ BSP coping skill ☐ Separation

Reason for Restraint (Imminent danger):

☐ Danger to Self ☐ Danger to Others ☐ Property destruction posing risk

Describe objectively: _____

Post-Restraint Medical Check (Within 1 hour):

Youth checked for injuries, breathing normally: ☐ Yes ☐ No (Seek medical attention)

Staff Signature: _____

Notifications:

On-Call QP Notified: ☐ Y ☐ N Time: _____ Guardian Notified: ☐ Y ☐ N Time: _____

IRIS Report Filed: ☐ Y ☐ N IRIS ID #: _____

Post-Restraint Debriefing (Within 24 hours):

Youth debriefed by: _____ Date/Time: _____

Trigger? _____

Youth alternative? _____

Staff alternative? _____

Youth Signature: _____ QP Signature: _____

4. Home Pass & Medicaid Billing Exclusion Tracker

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · Part 3 · Form 4: Home Pass & Medicaid Billing Exclusion Tracker

Youth: _____ Service Record # / MID: _____ Month/Year: _____

Date Left	Time Left	Destination / Pass	Date Returned	Time Returned	Total Hours Away	Billing Action (Suspension Days)	Staff Initials

Billing Coordinator Sign-Off: Medicaid billing adjusted for suspension days.

Signature: _____ Date: _____

5. Emergency Drill & Environmental Safety Log

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · Part 3 · Form 5: Emergency Drill & Environmental Safety Log

Facility: Well Spring Intervention LLC

Monthly Fire Drills (Under 3 minutes)

Month	Date	Time of Day	# Youth	Evac Time	Staff	Signature
Jan						
Feb						
Mar						
Apr						
May						
Jun						
Jul						
Aug						
Sep						
Oct						
Nov						
Dec						

Quarterly Tornado Drills

Quarter	Date	# Youth	Staff	Signature
Q1				
Q2				
Q3				
Q4				

Monthly Environmental Checks

Month	Smoke Detectors (Y/N)	Extinguisher (Y/N)	Hot Water ($\leq 120^{\circ}\text{F}$)	Fridge ($< 40^{\circ}\text{F}$)	Staff Initials
Jan					
Feb					
Mar					
Apr					
May					
Jun					
Jul					
Aug					
Sep					
Oct					
Nov					
Dec					

6. Employee SOP Acknowledgment

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · Part 3 · Form 6: Employee SOP Acknowledgment

Employee Name: _____

Title: ☐ QP ☐ AP ☐ Direct Care Professional

By signing below, I acknowledge that I have received, read, and understand the SOP Manual for **Well Spring Intervention LLC** (Rev. 2.4, July 2026, RMDM-Compliant). I understand these policies are mandated by NC DHSR (10A NCAC 27G), NC Medicaid (CCP 8C), Rule 108 (10A NCAC 27T), the NCDHHS Records Management and Documentation Manual (Effective July 8, 2025), NCGS Chapter 66 Article 40 (NC UETA) and the federal E-SIGN Act governing electronic signatures. I understand that the QP reports to the Clinical Director and is responsible for scheduling clinical services, assessments, PCPs, and day-to-day supervision of APs and DCPs according to the Clinical Director's direction, and provides recurring compliance reports to the Clinical Director as defined in §1.4 and §1.4(a). I acknowledge that QP credentialing requirements are specified in §1.4(b) per 10A NCAC 27G .0104 — a QP is not required to hold a full, unrestricted clinical license, but a bachelor's degree alone is not sufficient. I agree to follow these protocols exactly, including the electronic-signature safeguards and

system-unavailability procedures in §10.7. I understand failure to do so may result in disciplinary action, termination, or legal consequences regarding Medicaid fraud and regulatory non-compliance.

Employee Signature: _____ **Date:** _____

QP / Supervisor Signature: _____ **Date:** _____

7. Full Service Note Template (Mandatory)

Reference: RMDM Chapter 6 — Contents of a Full Service Note · Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · Part 3 · Form 7: Full Service Note Template

This template must be used for all shift notes. Backdating is prohibited. Photocopying or repeating notes verbatim from a prior date or another individual's record is strictly prohibited.

Field	Content
Youth Name	[Last, First]
Service Record # / MID	[Record # / MID]
Date of Service	[MM/DD/YYYY]
Service Name	Level 3 Residential — Shift
Type of Contact	[] In-person [] Telehealth [] Telephonic [] Collateral
Place of Service	[Facility Address / Location]
Shift / Coverage Hours	[e.g., Day Shift 7a-3p]
Staff Present (for ratios)	[List all staff names/titles on shift]
Purpose / ISP Goal Addressed	[Reference specific ISP/PCP goal # and objective]
Interventions Provided	[Objective, factual description of interventions, supports, activities, and de-escalation attempts if any]
Effectiveness & Youth Response	[Specific, individualized description of youth's response and progress toward goal]
Signature / Credentials / Date	[Full signature, credentials, and date authenticated]
Late Entry (if applicable)	[Late Entry made on ____ for service on ____]

8. Comprehensive Clinical Record Content Checklist

Reference: RMDM Chapter 2 — Full Clinical Service Records · Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · Part 3 · Form 8: Comprehensive Clinical Record Content Checklist

To be maintained in each youth's chart and reviewed quarterly by the QP. Mark each element Present (Y), Absent (N), or N/A, with date verified.

Element	Present? (Y/N/Date)	Notes
Treatment Consent		
PCP Consent		
Restrictive Intervention Consent (if applicable)		
Emergency Care Consent		
HIPAA Notice Acknowledgement		
Third-Party Release		
ROI(s)		
Demographics (name, DOB, contact, etc.)		
Emergency Contact / Preferred Physician / Hospital		
Advance Directives (or notation of none)		
Medication Allergies / Adverse Reactions / None		
Health / Behavioral Health History		
DSM-5-TR Diagnosis / ICD-10		
Medication Orders / MAR		
Lab Results (if applicable)		
Notification of Rights / Explanation		
Restrictive Intervention Documentation (if applicable)		
CCA (by licensed professional)		
ASAM Level of Care (if SUD)		
PCP / Service Plan / Crisis Plan		
Service Order (signed)		
Discharge Plan / Summary		
Accounting of Disclosures Log		
42 CFR 2.22 Summary (for SUD)		
Guardianship / POA / Legal Documents		
Incoming/Outgoing Correspondence		
Service Notes (per shift, compliant)		
Incident Occurrence Notations (reports filed separately)		

QP Quarterly Audit Signature: _____ **Date:** _____

9. Accounting of Disclosures Log

Reference: RMDM Chapter 3 — Documentation Requirements when Disclosing Information · Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · Part 3 · Form 9: Accounting of Disclosures Log

Maintain for minimum 6 years per HIPAA and RMDM requirements. One log per youth. All disclosures of SUD treatment information must additionally comply with 42 CFR Part 2.

Youth Name: _____ Service Record # / MID: _____

Date of Disclosure	Recipient (Agency/Individual)	Purpose of Disclosure	Description of Info Disclosed	Disclosing Staff (Name/Title)	Authorization / Exception Basis

10. Version History

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.4, Jul 2026 — RMDM-Compliant) · Part 3 · Version History

Versio n	Date	Summary of Changes	Author
1.0	Jan 2026	Original SOP.	Executive Director / QP
2.0	Jul 2026	Updated to full RMDM (July 2025) compliance: Added CDW/data reporting (§1.5); revised record retention to 12 years post-majority (§1.6); added CCA requirements & ASAM (§4.1); added medical necessity (§4.2); added service orders incl. verbal/72-hour (§4.4); added service authorization (§4.5); expanded service note content (§10.1-10.4); added alterations policy (§10.5); added authentication incl. initials/ADA/rubber stamps (§10.6); added administrative closure (§3.5); added accounting of disclosures (§11.4); added privacy/security/42 CFR Part 2 (§11); added individual access (§11.5); added transporting records (§11.6); added grids/modified notes (§10.4); added TB screening (§6.2); added comprehensive clinical record checklist (Form 8); added Full Service Note Template (Form 7); added Accounting of Disclosures Log (Form 9); added Protocol 20 (Service Orders & Auth) and Protocol 21 (Record Management).	Executive Director / QP

Version	Date	Summary of Changes	Author
2.1	Jul 2026	Auditor recommendation: Added §10.7 Electronic Signatures policy explicitly referencing NCGS Chapter 66, Article 40 (NC Uniform Electronic Transactions Act) and the federal E-SIGN Act (15 U.S.C. § 7001 et seq.). §10.7(a) specifies required administrative, technical, and physical safeguards (unique credentials, MFA, session timeout, TLS/encryption, immutable audit trails, RBAC, immediate revocation, annual review). §10.7(b) establishes System Unavailability Procedures, including paper-fallback handwritten signatures with date/credentials, QP-declared Documentation Continuity Event, 72-hour transcription into EHR with late-entry notation, paper source-document retention, 7-business-day QP review for delayed transcription, and a Documentation Continuity Event log. Prior §10.7 Service Authorizations and §10.8 Billing renumbered to §10.8 and §10.9 respectively.	Executive Director / QP
2.2	Jul 2026	Organizational clarification: §1.4 rewritten to separate the Clinical Director role (overall clinical program responsibility) from the Qualified Professional (QP) role. The QP now reports to the Clinical Director, supervises staff according to the Clinical Director's direction, and escalates clinical, staffing, and quality-of-care concerns. New §1.4(a) QP Compliance Reporting to the Clinical Director defines seven recurring compliance report types: monthly service-note audit summaries, monthly IRIS incident-report status, quarterly Clinical Record Content Checklist audits (Form 8), quarterly Accounting of Disclosures reviews (Form 9), quarterly personnel-file audits, annual electronic-signature safeguard review (§10.7(a)), and ad-hoc immediate reporting of breaches, complaints, licensing visits, and Medicaid audits. Clinical Director signs acknowledgment of each report and directs corrective action.	Executive Director / QP
2.3	Jul 2026	QP responsibilities clarification: §1.4 updated to explicitly include "scheduling" as a QP responsibility. The QP is now responsible for scheduling clinical services, assessments, and PCPs (in addition to clinical services, assessments, PCPs themselves and day-to-day AP/DCP supervision), according to the direction of the Clinical Director. Aligns the SOP with the operational reality that the QP coordinates and schedules all clinical appointments, assessment windows, and Person-Centered Plan meetings for each youth in care.	Executive Director / QP
2.4	Jul 2026	QP credentialing correction (10A NCAC 27G .0104): New §1.4(b) QP Credentialing Requirements added — specifies that a QP is NOT required to hold a full, unrestricted clinical license, but a bachelor's degree alone is not sufficient. Acceptable NC credentials listed across four categories: (i) full clinical license (LCSW, LPC, LMFT, Licensed Psychologist, LPA, psychiatrist MD/DO); (ii) associate/provisional license (LCSW-A, LPC-A, LMFT-A, LCAS-P); (iii) certification (LCAS, CCS with master's, CMSW); or (iv) psychiatric nursing credential (CNS or NP with psychiatric/mental health certification). Plus master's degree in human services, plus one year of full-time post-master's supervised experience, plus NC-DHHS QP training modules. QP must report any lapse/sanction/restriction to the Clinical Director within one business day. §2.2 Staff Qualifications QP bullet updated to match (previous bullet incorrectly stated "Master's or Bachelor's degree... licensed or license-eligible" — a bachelor's degree alone is not 27G .0104-compliant).	Executive Director / QP